CVS TESTBANK

Done by med 23:

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Embryology

Lecture-1

Embryo- normal development of the heart

1- The rough part of the left ventricle develops from:
A. Primitive ventricle
B. Proximal 1/3 of bulbus cordis
C. Sinus venosus
D. Conus cordi

Answer: A

Female edit slide 25

2- The vascular system develops at:
A. 2nd week
B. 3rd week
C. 4th week
D. 5th week

Answer: B

Female edit slide 11

3-Which of the following primitive vessels contributed to the development of the heart?
A. Left common cardinal
B. Right common cardinal
C. Left umbilical
D. Right umbilical
Answer: B

Female edit slide 29

(لأنة الوحيد الي يدخل في تصنيع جزء داخل القلب)

4-Which of the following helps with the development of the left auricle?
A. Pulmonary veins
B. Primitive atrium
C. Sinus venosus
D. Bulbus cordis

Answer: B

Female edit slide 30

5-Which of the following is the primitive structure of the coronary sinus?
A. Left sinus horn of sinus venosus
B. Right sinus horn of sinus venosus
C. Right vitelline vein
D. Right common cardinal vein

Answer: A

Female edit slide 29

6-Case about a person with heart enlargement, which structure is likely to grow downwards and outwards?
A. Apex
B. Left atrium
C. Right ventricle
D. Pulmonary trunk

Answer: A

Not sure if it embryo

7-A newborn has a defective coronary sinus, and the cause is most likely congenital. Which part of the primitive heart tube is most likely responsible for the defect?
A. Sinus venosus
B. Primitive atrium
C. Primitive ventricle
D. Bulbus cordis

Answer: A
Female edit slide 29

8- Which of the following primitive structures develops into the left auricle?
A. Primitive atrium
B. Bulbus cordis
C. Pulmonary veins
D. Sinus venosus

Answer : A

Female edit slide25

9- The vestibule of the left ventricle is formed from?
A. Sinus venosus
B. Primitive atrium
C. Primitive ventricle
D. Bulbus cordis

Answer: D

Female edit slids

10-A patient has a thin outflow part of the right atrium. What is the most likely defect?
A. Body & right sinus horn
B. Conus cordis
C. Primitive ventricle
D. Sinus venosus

Answer: A
Female edit slide 30

Lecture-2

Embryo - abnormal development of the heart

1-After birth, the patent ductus arteriosis is an abdominal connection between:

A.Arch of aorta and pulmonary trunk
B.Right and left atrium
C.Right and left ventricle
D.Ascending aorta and pulmonary trunk

Answer:A
Female edit slide 35
(الدكتورة قالت انه يمسك في ال descending بي لو مو موجود ذا الخيار يعتبر اصح)

2-ostium secundum is a defect in:

A) Upper part of septum primum
B) Lower part of septum primuim
C) Upper part of septum secundum
D) Lower part of septum secundum

Answer:A
Female edit slide 12
(مو مذكورة نصا بس بالفهم تنحل)

3-with potent ductus arteriosis the blood :

A.Moves from pulmonary trunk to aortic arch
B.Moves from pulmonary trunk to ascending aortic
C.Moves from umbilical vein to SVC
D.Moves from umbilical artery to placenta

Answer:A
Female edit slide 32

4-which of the following is not included in the tetralogy of Fallot?

A.Overriding aorta

B.Hypertrophy of right ventricle

C.Atrial septal defect

D.Pulmonary stenosis

Answer:C
Female edit slide 23

5- A patient was found to have their aorta originating from the right ventricle and the pulmonary arteries originating from the left. What is the name of this defect?

A. Defect in the interatrial septa
B.Transposition of the great vessels
C.Tetralogy of Fallot
D. Persistent truncus arteriosus

Answer:B
Female edit slide 22

6-defect in the heart that cause overrides both ventricles and receives blood from both sides what is the reason?

A.Interventricular septal fail to develop
B.Conotruncal septal fail to develop
C.Valvular defect
D.open ductus arteriosus

Answer:B
Female edit slide 21

7-what structure can cause Atrial septal defects (ASD) through the opening of?
A.Foramen ovale
B.Patent ductus venosus
C.Patent ductus arteriosus
D.Persistence of truncus arteriosus.

Answer:A
Female edit slide 11

8-Which of the following embryonic structures allows fetal blood to circulate from the pulmonary trunk to the descending aorta before birth?
A. Ostium primum
B. Ostium secundum

C. Ductus arteriosus
D. Ductus venosus

Answer: C
Female edit slide 31

9-A child presents to the ER, and after a series of procedures, you determine that his aorta is on the right side of his pulmonary trunk. What is the cause behind this defect?
A. Failure of conotruncal septum to follow its spiral course
B. Unequal division of the conus
C. Failure of conotruncal ridges to fuse
D. Atrial septal defect

Answer: A
Female edit slide 22

10-Pulmonary stenosis, VSD, overriding aorta, and ventricular hypertrophy are characteristic of which of the following?
A. Tetralogy of Fallot
B. Transposition of great arteries
C. Atrial septal defect
D. Patent ductus arteriosus

Answer: A
Female edit slide 23

11- A patient has ventricles that share one outflow tract. What is the most likely defect?
A. Ventricular septal defect
B. Conotruncal septum not formed
C. Incomplete development of endocardial cushions
D. Persistent truncus arteriosus

Answer: D
Female edit slide 21

12-Failure of the conotruncal ridge to twist results in which condition?

A. Transposition of the great vessels

B. Ventricular septal defect

C. Atrial septal defect

D. Persistent truncus arteriosus

Answer: A

Female edit slide 22

13-Which embryonic structure closes to form the ligamentum teres?

A. Umbilical vein

B. Distal part of umbilical arteries

C. Proximal part of umbilical arteries

D. Vitelline vein

Answer: A

Female edit slide36

ANATOMY-SDL

SDL anatomy-radiology

1. The imaging technique used to follow up a patient with aortic valve stenosis is:

A. Echocardiograph
B. MRI
C. Angiography
D. X-ray of skull

ANSWER: A

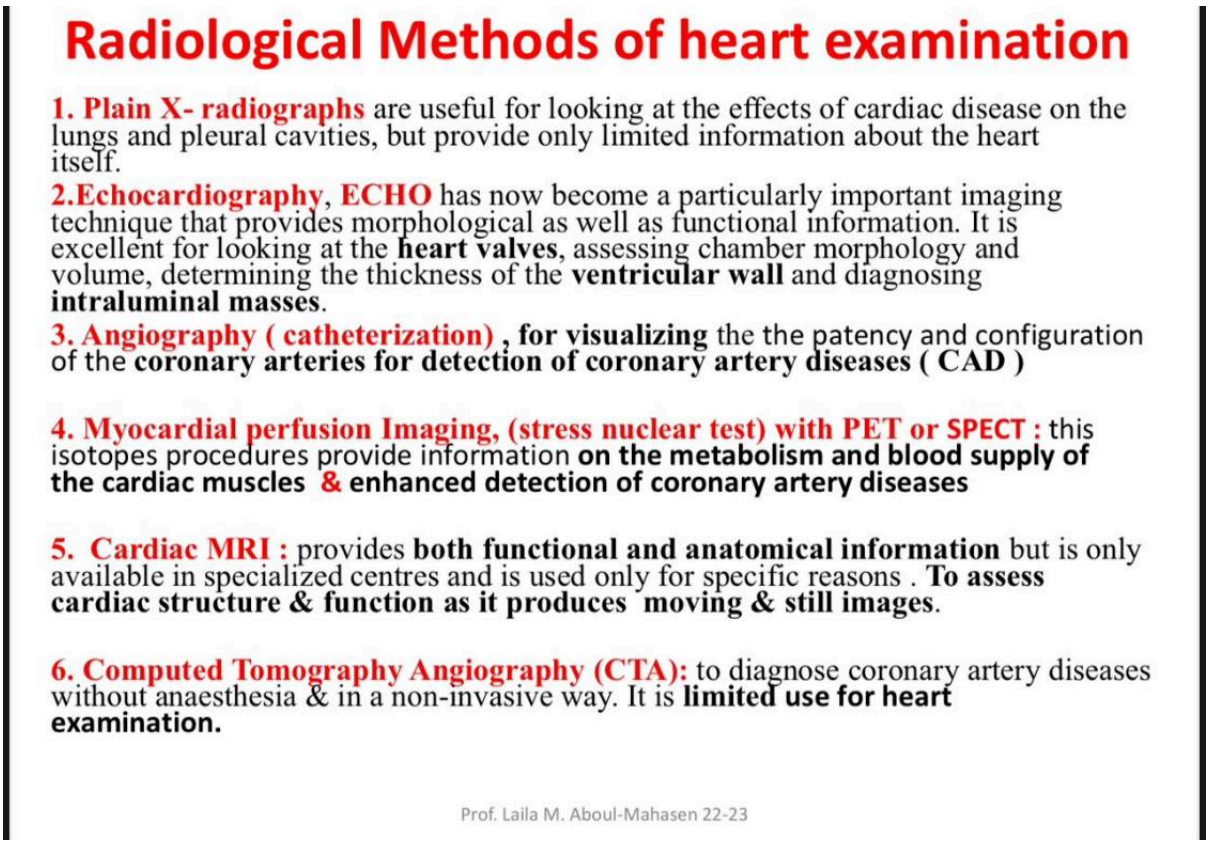
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2. A newly born baby is born cyanotic and having difficulty breathing. On examination, a well-heard diastolic murmur suggests valvular stenosis. The imaging technique used is:

A. Echocardiogram
B. MRI
C. Angiography
D. X-ray of the skull

ANSWER: A

الأسئلة غالبا ماحتطلع من ذي السلايدة



Radiological Methods of heart examination

1. Plain X- radiographs are useful for looking at the effects of cardiac disease on the lungs and pleural cavities, but provide only limited information about the heart itself.

2.Echocardiography, ECHO has now become a particularly important imaging technique that provides morphological as well as functional information. It is excellent for looking at the **heart valves**, assessing chamber morphology and volume, determining the thickness of the **ventricular wall** and diagnosing **intraluminal masses**.

3. Angiography (catheterization) , for visualizing the the patency and configuration of the **coronary arteries for detection of coronary artery diseases (CAD)**

4. Myocardial perfusion Imaging, (stress nuclear test) with PET or SPECT : this isotopes procedures provide information **on the metabolism and blood supply of the cardiac muscles & enhanced detection of coronary artery diseases**

5. Cardiac MRI : provides **both functional and anatomical information** but is only available in specialized centres and is used only for specific reasons . **To assess cardiac structure & function as it produces moving & still images.**

6. Computed Tomography Angiography (CTA): to diagnose coronary artery diseases without anaesthesia & in a non-invasive way. It is **limited use for heart examination.**

Prof. Laila M. Aboul-Mahasen 22-23

Physiology

L8- Physiology,Normal ECG

1- What view of the heart does lead II, III, and , AVF represent?

A. Inferior
B. Anterior
C. Lateral
D. Septal

Ans: A

Female edited slide 29

2- Lead 2 is :

A. +ve in Right arm and -ve in left arm
B. +ve in Right arm and -ve in left leg
C. +ve in Left leg and -ve in Right arm
D. +ve in Left leg and -ve in Left arm

Ans : C

Female edited slide 8

3- What wave in the ECG represents repolarization of both ventricles?

A. T wave
B. QRS Complex
C. P wave
D. P-R interval

Ans : A

Female edited slide 13

4- Which of the following is correct regarding the electrical impulse that generates from the SA node as seen on an ECG?

A. QRS complex (0.8 second)
B. PR interval (0.12-0.2 seconds)
C. T wave (0.25 second)
D. QT interval (0.3-0.4 seconds)

Ans: B
Female edited slide 14

5- What wave in the ECG represents ventricular repolarization?

A. Q wave
B. R wave
C. T wave
D. P wave

Ans: C
Female edited slide 13

6- The ventricle becomes completely depolarized in ECG accompanied with the plateau in which of the following phases ?

A. QRS complex
B. QT interval
C. ST segment
D. T wave

Ans : C
Female edited slide 22

7- Ventricular repolarization in ECG is represented by?

A. QRS complex
B. P wave
C. T wave
D. U wave

Ans: C
Female edited slide 13

8- In a healthy heart, the QRS waves correspond to?

A. Ventricular repolarization
B. Ventricular depolarization
C. Atrial repolarization
D. Atrial depolarization

Ans: B
Female edited slide 12

9- A patient has an elevated ST segment and inverted T wave. What is the likely diagnosis?

A. Acute MI (ischemic heart disease)
B. Atrial fibrillation
C. Myocardial ischemia
D. Pulmonary embolism

Ans: A
female edited slide 28

10- What indicates QRS in ECG?

A. ventricular depolarization
B. ventricular repolarization
C. plateau
D. Atrial depolarization

Ans : A
Female edited slide 12

11- ST elevated in the ECG indicates that the patient has ?

A. Myocardial infarction
B. Myocardial ischemia
C. Tachycardia
D. Atrial Fibrillation

Ans :A

Female edited slide 28



12- What does the ECG show ?

A. The ECG shows elevated S-T segment
B. The ECG shows elevated T wave
C. The ECG shows prolonged P-R interval
D. The ESG shows inverted S-T segment

Ans: A

Female edited slide 28

L9 - Physiology, Cardiac Arrhythmias

1- A patient was given an ACE inhibitor and developed arrhythmia. What is the most likely ion imbalance causing this, given that a tall, peaked T-wave is visible on the ECG?

A. Hyperkalemia
B. Hypokalemia
C. Hypercalcemia
D. Hypocalcemia

And : A

Female edited slide 28

2- In first-degree heart block what can be expected ?

A. PR interval is less than 0.12 (3 small squares)
B. PR interval is prolonged
C. P wave is sometimes followed by QRS
D. P wave is not followed by QRS

Ans : B

Female edited slide 22

3- What make Narrow and tall peaked T wave in ECG?

A. Hyperkalemia
B. Hypokalemia
C. Hypocalcemia
D. Hypercalcemia

Ans :A

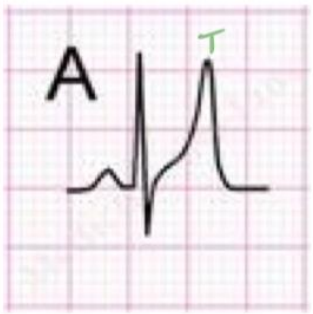
Female slide 28

4- A patient is noted to have an abnormally shortened PR interval on their ECG. What could be the cause?

A. Atrial fibrillation
B. Right bundle branch block (RBBB)
C. Left bundle branch block (LBBB)
D. Wolff-Parkinson-White (WPW) syndrome

Ans : D

Female edited slide 26



5- What is the cause of the ecg in the picture above?

A. Hyperkalemia
B. Hypokalemia
C. Hypercalcemia
D. Hypocalcemia

Ans: A

Female edited slide 28

6- An ECG shows no P-waves and an irregular rhythm. What does this represent?

A. Atrial fibrillation
B. Sinus tachycardia
C. Ventricular extrasystole
D. Ischemia

Ans:A

Female edited slide 19

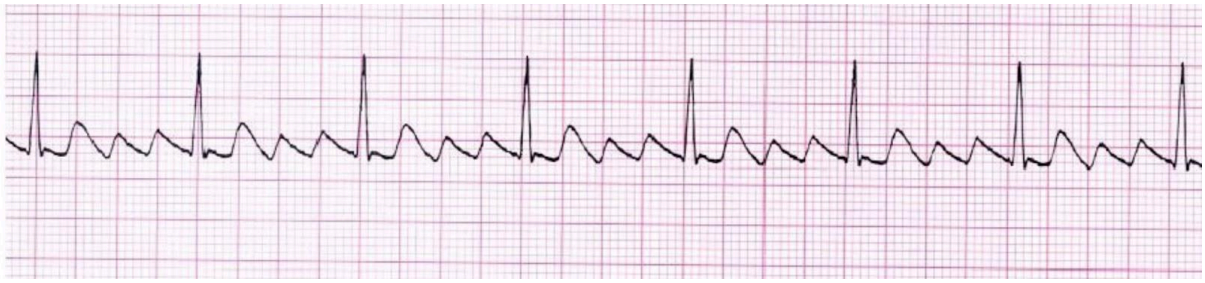
7- In ECG tall Peaked T-wave with “tent” like appearance indicates?

A. Hyperkalemia
B. Hypokalemia
C. Hypercalcemia
D. Hypocalcemia

Ans: A

Female edited slide 28

8- Which ECG shows ventricular extrasystole?

A. 

B. 

C. 

D. 

Ans : C

Female edited slide 15

Physiology: Cardiac cycle

1. Which one of the pairing statements is correct?

A. First heart sound - closure of semilunar valves
B. 3rd heart sound - elderly people
C. Isovolumetric relaxation phase - sharp decrease in ventricular pressure
D. Right Atrial systole - passive filling of the ventricle

ANSWER: C

Female slide 14, slide 17 line D to A

2. What will happen at the beginning of the isovolumetric contraction?

A. Aortic pressure increased
B. Aorta expansion
C. Ventricle volume decreased
D. A-V valves closure

ANSWER: D

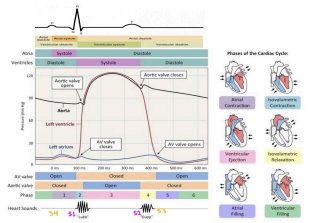
Female slide 11

3. Regarding Ventricular Systole?

A. Depends on intracellular Ca++
B. Ends by the end of plateau
C. Corresponds P wave in the ECG
D. It occurs after atrial diastole

ANSWER: B

Med 23 Male edited slide 20



4.When the ventricular pressure is lower than the atrial pressure this leads to:

A. AV valve open
B. Aortic outflow of the blood
C. Right ventricular contraction
D. Semilunar valve open

ANSWER: A

Female slide 15 (7. Rapid filling phase)

5.Between AV valves closure and opening of Semilunar valves is the following phase:

A. Isometric relaxation
B. Rapid ejection
C. Isometric contraction
D. Reduced ejection

ANSWER: C

Female slide 11,12

6. The lowest ventricular volume occurs during which phase?

A. Isovolumetric ventricular relaxation
B. Isovolumetric ventricular contraction
C. Rapid ventricular ejection
D. Reduced ventricular ejection

ANSWER: A

Female slide 17 Line D to A

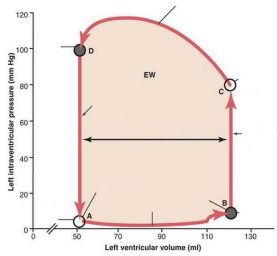
7. QRS complex of ECG occurs immediately before which phase?

A. Isometric contraction
B. Isometric relaxation
C. Atrial systole
D. Maximum filling phase

ANSWER: A

Female slide 10

8. Which line represents the :



Isovolumetric relaxation phase ?

A. Line A-B
B. Line B-C
C. Line C-D
D. Line D-A

ANSWER: D

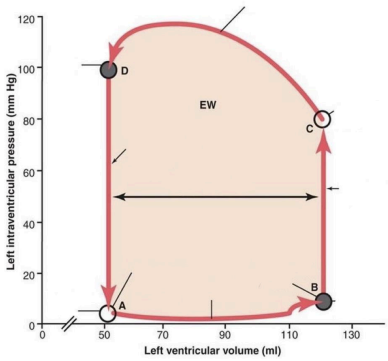
Female slide 17,18

9. At which point
the closing of the mitral valve will occur ?

A. A
B. B
C. C
D. D

ANSWER: B

Female slide 18

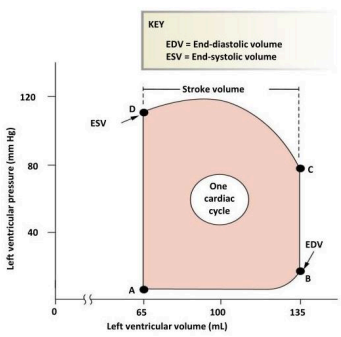


10. Which line represents the **isovolumetric relaxation** phase in the pressure-volume loop of the left ventricle?

A. Line D-A
B. Line B-C
C. Line C-D
D. Line A-B

ANSWER: A

Female slide 17



11. In the ejection phase, the atrioventricular valves ……,and the semilunar valves…… .

A. Close, open
B. Close, close
C. Open, close
D. Open, open

ANSWER: A

Female slide 12

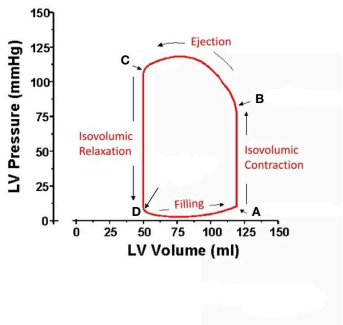
12. The phase of the cardiac cycle between mitral valve closure and aortic valve opening?

A. Isovolumetric contraction phase
B. Atrial systole
C. Ventricular systole
D. Isovolumetric relaxation phase

ANSWER: A

Female slide 18

13. Point A represents?



A. Mitral valve closure
B. Mitral valve opening
C. Aortic valve opening
D. Aortic valve closure

ANSWER: A

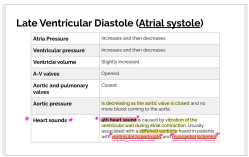
Female slide 18

14. On cardiac examination, the patient's rhythm was normal with S4 at the apex. What characterizes the S4 sound?

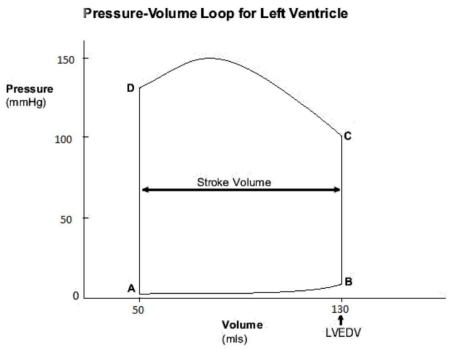
A. It is caused by atrial relaxation during ventricular ejection
B. It can be heard in patients with left ventricular hypertrophy
C. It can be heard with stethoscope in the diaphragm
D. It has a high frequency, usually 600 cycles/sec

ANSWER: B

Med 23 Male edited slide 10



15. At which point does the mitral valve closure occur?



A. B
B. A
C. D
D. C

ANSWER: A
Female slide 18

16. The graph shows ventricle pressure-volume relationship; point A is:

ANSWER: Mitral valve closure

17. Reduced ejection fraction occurs in which of the following ?

A. Systolic dysfunction

B. Diastolic dysfunction

C. Exercise

D. Normal regulation of blood pressure

ANSWER: A

Female slide

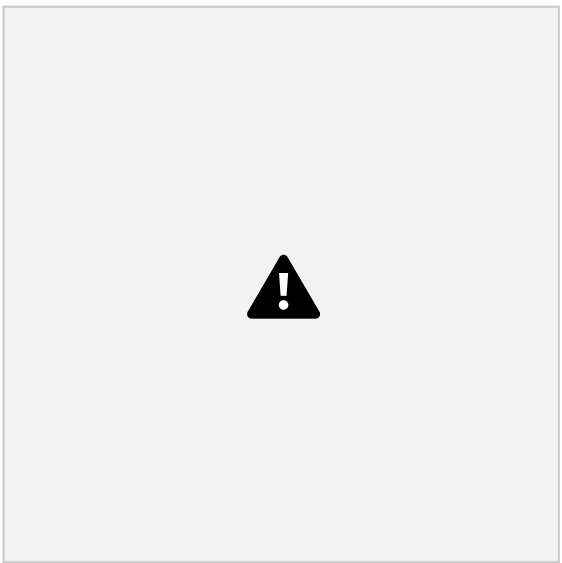
18. If there was an infarction of the AV node that caused an abnormal increase in the AV nodal delay, what will this affect?

A. Ventricular filling
B. Isovolumeric relaxation
C. Atrial systole
D. Ventricular systole

ANSWER: A

Female slide

19. Which point represents mitral valve closure ?



A. A

B. B
C. C
D. D

ANSWER: B

Female slide

20. Arteries are elastic and recoil during diastole, why?

A. To maintain blood pressure
B. To maintain constant blood flow
C. To maintain blood volume
D. For exchange

ANSWER: B

Female slide 23 (The arterial pulse curve)

L11 - Coronary circulation:

1. Coronary flow is mainly regulated by:

A. Metabolic regulation
B. Auto regulation
C. Nervous regulation
D. Mechanical regulation

ANSWER: B

Female slide 10

2. A medical student measured the effect of sympathetic stimulation on coronary blood flow. He created two groups: Group A and Group B. He administered an alpha blocker to Group A and a beta-1 blocker to Group B. What are the results of the sympathetic effect on coronary blood flow (CBF)?

A. No difference between the two groups
B. Increase in group A
C. Increase in group B
D. Decrease in group A

Answer : B

Female slide 10

3. What is true about coronary circulation?

A. The heart gets a rich blood supply from the right and left coronary arteries that arise from the coronary sinus
B. During ventricular systole the heart presses the arteries causing them to obstruct
C. Ventricles only fill during diastole

Answer : ?

Female slide ?

4. A medical student wants to measure the effects of sympathetic stimulation on coronary blood flow. He made two groups of rats. In Group A, he uses a beta blocker, and in Group B, he uses an alpha blocker. The sympathetic effect:

A. Increase in group B
B. Increase in group A
C. Decrease in group B
D. No change in both groups

Answer : A (increase in group B)

Female slide 10

5. Left coronary blood flow is maximal in?

A) Early diastole
B) Late diastole
C) Early systole
D) Late systole

ANSWER: A

Female slide 7

6. Left coronary arterial blood flow is maximal in:

Answer: Early diastole

Female slide 7

7. Which of the following is not a classic ECG change in MI:

A. Double P wave

B. Inverted T wave
C. ST elevation
D. Abnormal development of deep Q wave

Answer : A

Female slide 16

8. At rest, the heart extracts up to …. of Oxygen

A. 100%
B. 75%
C. 50%
D. 25%

Answer : B

Female slide 9

9. Which of the following is NOT a classic change in ECG in myocardial infarction?

A. Inverted T wave
B. ST segment elevation
C. Development of an abnormal Q wave
D. Double P wave

Answer : D

Female slide 16

L: Physiology of microcirculation & cardiac edema

1) An increase in capillary hydrostatic pressure can be caused by :

A) Decrease venous resistance
B) Decrease venous return
C) Arteriolar vasodilation
D) Arteriolar vasoconstriction

ANSWER: C

Female Edit slide 17

2) Edema in congestive heart failure is due to :

A) Increase vascular permeability
B) Increase hydrostatic pressure
C) Decreased oncotic pressure
D) lymphatic obstruction

ANSWER: B

Female Edit slide 20

3) A 19-year-old man has a severed artery and is experiencing slight hypotension after first aid, along with a reactive pupil. What is the greatest redistribution of blood volume?

A. Arteries

B. Arterioles

C. Veins

D. Capillaries

ANSWER: C

Female Edit slide ?

4) A Patient with a hydrostatic pressure 25 and osmotic capillary pressure of 10, and osmotic colloid pressure of 30 and capillary hydrostatic pressure 3,

What is the compelete capillary net pressure ?

Answer: Filtration pressure up by 2

Female Edit slide 10, 13, 14

5) Which of the following changes results from decreased level of plasma proteins following a chronic liver disease?

A. Decreased capillary hydrostatic pressure
B. Increased capillary hydrostatic pressure
C. Decreased colloid osmotic pressure
D. Increased colloid osmotic pressure

Answer: C

Female Edit slide 12

6) After an accident, a patient lost around 1.5 L of blood. Which one would probably be increased?
A. Capillary oncotic pressure

B. Interstitial oncotic pressure
C. Number of opened capillaries
D. Fluid movement to the interstitium

Answer: A

Female Edit slide 13

7) A patient who has dialysis three times a week and has edema in both legs is likely experiencing:

A. Reduced Plasma Oncotic Pressure
B. Increased Capillary Hydrostatic Pressure
C. Increase Plasma Oncotic Pressure
D. Decreased Capillary Hydrostatic Pressure

Answer :A

Female Edit slide 19, 20

8) Venous end of capillary favours

A. Filtration
B. Reabsorption
C. Inc capillary hydrostatic pressure

Answer :B
Female Edit slide 17

9) Albumin-urea occur in which type of edema :

A. Lymphatic obstruction
B. Pregnancy
C. Nephrotic syndromes

ANSWER: C

Female Edit slide 12

10) Hydrostatic pressure increases in which of the following :

A. Arterial dilatation
B. Arterial constriction

ANSWER: A

Female slide 17

11. Local blood flow in small arterioles contraction due to an increase in stretch in the arteriolar wall. This leads to:

A. Increased blood flow to tissue in response to increased arterial blood pressure (ABP)

B. Increased blood flow to tissue in response to decreased arterial blood pressure (ABP)

C. Keeping blood flow constant while blood pressure rises

D. Decreased blood flow to tissue in response to increased arterial blood pressure (ABP)

Answer: C

Female Edit slide

1. All are symptoms of hypovolemic shock except :

A) Thirst sensation
B) Tachypnea
C) Increase in temperature
D) Anuria

ANSWER: C

2. A compensatory mechanism of acute hypovolemic shock results in:

A) Inhibition of impulses from the arterial baroreceptors
B) Decrease renin release
C) Tachycardia

ANSWER: C

3. A women in restaurant, after she start eating her meal she became anxious, itching and wheezing, feels like her throat is closing up, now she is hypotensive and have tachycardia, the following description mimics which type of the following:

A) Neurogenic shock
B) Hypovolemic shock
C) Anaphylactic shock
D) Cardiogenic shock

ANSWER: C

4. A 37 year old came to the clinic with shock. She was diagnosed with anaphylactic shock rather than hypovolemic shock mainly because of a decrease of which of the following variables:

A. TPR
B. HR
C. CO

D. Creatinine kinase

5. 76-year old man was suffering of vomiting and diarrhea recently he has been feeling tired and after examination it was revealed he is dehydrated, his pulse was 127 bp and his pressure was low, which of the following is the diagnosis?

A. Cardiogenic shock
B. Hypovolemic shock
C. Septic shock
D. Anaphylactic shock

Answer: B

6. patient devolved shock and sever vasodilation due to release of histamine after take aspirin what type of shock?

A. Cardiogenic
B. Distributive
C. Hypovolemic

Answer :B

7. A patient had a moderate hemorrhage. Which of the following is a compensatory mechanism to hemorrhage?

A. Stroke Volume
B. Venous Capacitance
C. Renal and Splanchnic Resistance
D. Heart Rate (HR)

Answer :C

8. A patient had a severe duodenal ulcer and lost 600 ml of blood in 1 hour. What mechanism will occur?

A. Increase Atrial Mean Pressure and Decrease Cardiac Output

B. Decrease Central Venous Pressure and Increase Cardiac Output

C. Decrease Central Venous Pressure and Decrease Cardiac Output

Answer: C

Biochemistry

L3- Biochemistry,Metabolism of Cholesterol

1- Inhibiting the rate limiting enzyme of cholesterol biosynthesis would be beneficial in lowering blood cholesterol levels by which of the following mechanisms ?

A. Preventing reabsorption of bile acid
B. Increasing the conversion of cholesterol to bile acid
C. Inhibiting the formation of mevalonate
D. Inhibiting the formation of HMG coA

Ans: C
Famle edited slides 11 & 13

2- 40 years old obese women presented with recurrent episodes of right upper quadrant pain radiating to the back, it was found that she has gallstones, what is the most likely cause ?

A. Increase cholesterol secretion into the bile
B. Increase secretion of phospholipid to bile
C. Increase secretion of bile salt to bile
D. More bile than phospholipids

Ans: A
Female edited slide 17

3- Decreasing the rate-limiting enzyme for cholesterol synthesis would be beneficial for decreasing cholesterol levels by which mechanical?

A . Increasing conversations of cholesterol to bile acids
B. Preventing reabsorption of bile acid in intestine
C.. Inhibiting the synthesis of mevalonate
D. Inhibiting the absorption of cholesterol from diet

Ans:C
Female edited slides 11&13

4- The anticholesterolemic action of simvastatin is based on its effectiveness as a

competitive Inhibitor of the rate limiting enzyme in cholesterol biosynthesis. The reaction product normally produced by this enzyme is?

A. Squalene
B. Methylmelonate
C. Lanosterol
D. Mevalonate

Ans: D

Female edited slide 23

5- What is the subcellular site of synthesis of cholesterol?

A. Mitochondria
B. Cytoplasm
C. Nucleus
D. Golgi apparatus

Ans: B
Female edited slide 14

6- What is the intermediate molecule between ketone and cholesterol synthesis?

A. Citrate
B. HMG-CoA
C. Mevalonic acid
D. Acetoacetate

Ans: B
Female edited slide 14

7- HMG coA reductase is a dephosphorylation enzyme. Which of the following hormones activate it by dephosphorylation ?

A. Insulin
B. Glucagon
C. LPL
D. Epinephrine

Ans : A

Female edited slide 22 & 23

8- A patient has hypercholesteremia. One of the ways to decrease cholesterol level is to inhibit the rate limiting enzyme in cholesterol biosynthesis. What is the mechanism?

A. Prevent the formation of Mevalonate
B. Increase the activity of HMG-CoA reductase
C. Stimulate the production of LDL receptors
D. Enhance cholesterol absorption from the intestines

Ans : A

Female edited slide 11&13&14

L4- Biochemistry, Diagnostic cardiac markers

1- A 62-year- old man is administered to the ER due to a myocardial infarction. Four days later after admission he had another chest pain. What is the best cardiac enzyme to investigate ?

A. Troponin I
B. Ck-mb
C. Troponin T
D. LDH

Ans:B
Female edited slide 18

2- What is the best cardiac enzyme for risk stratification and prognosis assessment for acute coronary syndrome (ACS) ?

A. Ck-MB
B. Troponin
C. LDH
D. Myoglobin

Ans: B
Female edited slide 11

3- The cardiac marker most specific for MI is?

A. Total CK
B. Myoglobin
C. LDH
D. Troponin

Ans: D
Female edited slide 11

4- Which of the statements are correct about cardiac markers?

A. Troponin L,C,I are all cardiac markers
B. Myoglobin more sensitive and use early diagnosis of myocardial infarction
C. Lactate dehydrogenase (LDH) is the most specific marker for myocardial infarction
D. Creatine kinase (CK) levels rise only after myocardial infarction and are not influenced by other conditions

Ans: B

Female edited slide 19

5- Which type of the following markers is the earliest marker?

A. Cardiac troponin
B. CK-MB
C. Myoglobin
D. D-BNP

Ans: C
Female edited slide 19

6- A patient came to the clinic complaining from chest pain that started a week ago. Which of the following markers would you measure?

A. BNP
B. Troponin
C. Creatine kinase
D. Myoglobin

Ans : B
Female edited slide 13

7- Troponin appears in the blood after the onset of a myocardial infarction (MI) and remains elevated for?

A. 3 hours, 10 days
B. 5 hours, 2 days
C. 10 minutes, 12 days
D. 10 hours, 5 days

Ans: A

female edited slide 13

8- A farmer has been experiencing muscle pain in the limbs, especially the shoulders and thighs, expected biochemical finding?

A. Elevated CK-MM
B. Elevated CK-MB
C. Low myoglobin
D. Elevated CK-BB

Ans : A
Female edited slide 16

9- Which biomarker determines congestive heart failure?

A. BNP
B. Myoglobin
C. Troponin
D. Creatine kinase

Ans: A
Female edited slide 23

10- Eliza a 66-year- old patient came to the ER with a chest pain that has been progressively increasing for the past 12 hours what are the 2 cardiac markers they may be at their peak

A. Troponin and CK-MB
B. CK-MB and myoglobin
C. Troponin and BNP
D. Troponin and myoglobin

Ans: A
Female edited slide 13&18

11- A patient came to the clinic complaining of chest pain that started one week ago. Which of the following biomarkers would you measure?
A. CK-MB
B. Myoglobin
C. Troponin
D. BNP

Ans: C
Female edited slide 13

12- A patient experienced chest pain due to a myocardial infarction (MI) four days ago. One week later, the patient feels chest pain again. Which biomarker is most likely to be elevated at this time?

A. Troponin-T
B. Troponin-I
C. CK-MB
D. LDH

Ans: both A&B
Female edited slide 13

13- What is the ideal cardiac marker?
A. Present at high concentration in non-myocardium
B. Present soon after onset of ischemia
C. Present late after the ischemia
D. Present at low concentration on myocardium

Ans:B
Female edited slide 7

Pathology

Ischemic heart diseases 1 and 2

24 - Patient came with chest pain ecg showed myocardial ischemia, and cardiac enzyme was high?
a. prolonged ischemia with cardiac myocyte necrosis
b. prolonged ischemia with myocardial fibrosis
c. transient ischemia with myocardial fibrosis
d. transient ischemia with necrosis

Ans: A
Female slide 4

41- what is the most common cause of sudden cardiac death?
a. Ventricular fibrillation
b. contracctile dysfunction
c. papillary muscles dysfunction
d. Mitral valve stenosis

Ans: A
Female slide 19

23- Which of the following is best defined the sudden cardiac death?
a. Death within 24 hrs since the start of symptoms
b. Death within 2 days since the start of symptoms
c. Cardiac death after prolonged ischmia
d. Death within 1 week since the start of symptoms

Ans: A
Female slide 18

45) Which of the following best describes the difference between ischemia and infarction?
a. Ischemia is reversable, Infarction is not
b. Ischemia causes cell death, Infarction does not
c. Infarction is reversible, Ischemia is not
d.Ischemia and Infarction are different terms for the same condition

Ans: A
Female Slide 5

17) Prinzmetal Angina is characterized by chest pain during rest or sleeping, what is the pathological state of the coronary artery ?

a. Coronary artery Vasculitis
b. Coronary artery partial thrombus
c. Coronary artery spasm near plaque
d. Ischemic thrombus

Ans: C
Edited Female Slide 17

2- Which statement is accurate about stable angina ?

A. Induced by exertion, relieved at rest
B. Induced by exertion, does not respond to nitroglycerine
C. Starts at rest, relieved by nitroglycerine
D. Starts at rest, relieved by oxygen mask

Ans: A
Edited Female Slide 16

36- Characteristic of a stable plaque?

A. Large lipid in core
B. Thick fibrous cap
C. Thin fibrous cap
D. No fibrous cap

Ans: B
Edited Female Slide 7

37- An elderly man with chest pain that comes when they walk up the stairs was given sublingual nitroglycerine that relieves his symptom. What is the pathological mechanism of his condition?

A. Transient ischemia due to increased oxygen demand
B. Fixed coronary obstruction
C. Coronary vasospasm
D. ischemia due thrombosis

Ans: A
Edited Female Slide 16

23- 40y old male came with chest pain occur at rest , serum show cardiac enzymes normal , what the best explain in this situation ?
a- removed by liver rapidly
b- myocyte membrane is still intact
c- necrosis
d- complete occlusion of coronary artery

Ans: B
(Because the cell has not died yet, the enzyme has not been released)

55. Patient with infarction in the anterior surface of left ventricle and anterior IVS, which coronary artery is most occluded?
A- Anterior inter-ventricular artery
B- Posterior inter-ventricular artery
C- Circumflex artery
D- Right coronary artery

Ans: A
Edited Female Slide 9

41- A patient came to the ER with chest pain, they have a history of uncontrolled hypertension, they died, most likely cause of death?
A- Fibrosis of heart tissue
B- Reversible injury
C- Necrosis
D- Angina

Ans: C
(angina = reversible injury will not lead to death, necrosis is irreversible will lead to death)

42- A patient has MI, according to the coronary artery supply of the heart, which part of the heart is most likely affected?

A. Anterior left ventricle wall + interventricular septum
B. Posterior ventricle wall + interventricular septum
C. Posterior ventricle wall + lateral wall
D. Lateral wall + interventricular septum

Ans: A
Edited Female Slide 9

43- Patient came in with acute MI and died, the most likely cause of death?

A. Arrhythmia
B. ventricular hypertrophy
C. Heart failure
D. Mitral valve stenosis

Ans: A
Edited Female Slide 11

18- An autopsy the heart of a 65 years old woman showed ruptured left ventricular aneurysm. What is the most probable pathogenesis of this disease?
a. Verrucous endocarditis
b. Healed fibrosis pericardium
c. Healed myocardial infarction
d. Myocardial inflammation

Ans: C
Edited Female Slide 12

33) A patient is suffering from intermittent pain in his chest that is related to effort and its relieved by nitroglycerin. What are the changes happened to the heart?
a. Reversable ischemia
b. Necrosis
c. Irreversable ischemia
d. Fibrosis

Ans: A
Female Slide 13

29- What is the most common cause of ischemic heart disease?

a. Infection
b. Vasculitis
c. Atherosclerosis
d. Embolism

Ans: C
Edited Female Slide 3

34- patient came with chest pain for 2 years and he treated by nitrate but the pain isn't resolved anymore and let him wake from sleeping. What type of ischemia

a. uncomplicated atherosclerosia
b. unstable ATH with ulcerated partial thromb
c. stable ATH with no ruptured
d. Typical angina

Ans: B
Edited Female Slide 18
(D + C + A will be treated by nitrate)

Valvular heart diseases

15 years old boy had erythematosus rash in the abdomen and fever , after 2-3 weeks rise rashe devolpe, Joint pain with holosysolic murmur , What is his condition?

a- Aschoff bodies in the valve

b- thrombosis in valve

c- infectied endocarditis

Ans: A

Female slide 12

1) What is the most common valvular lesion in chronic rheumatic disease?

a. Aortic regurgitation

b. Aortic stenosis

c. Mitral stenosis

d. Mitral regurgitation

Ans: C

Female slide 25

17-year-old boy has a bacterial infection that developed after 2 weeks of fever, Sydenham chorea involuntary movement, and friction rub what is the best serum diagnosis?

a- anti-endothelial ab

b- Antistreptolysin O

c- Antistreptolysin A

ANS: A

Female slide 33

10-year-old girl develops subcutaneous nodules over the skin of her arms 3 weeks after a an of acute attack of acute pharyngitis . She manifests choreiform involuntary movements and begins to complain of pain in her knees and hips. A friction rub is heard on auscultation of her chest. which of the following laboratory findings is most characteristic of the disease affecting this girl?

a. Elevation Anti-streptolysin O antibody titer

b. Elevation Antinuclear antibody titer

c. Elevation Creatinine level

d. Elevation Troponin level

ANS: A

Female slide 33

What is the nature of anitschow cells ?

A) Activated macrophage

B) T lymphocyte

C) Plump neutrophils

D) mast cells

ANS: A

Female slide 14

10-year-old girl develops subcutaneous nodules over the skin of her arms and torso 3 weeks after a bout of acute pharyngitis. She manifests choreiform movements and begins to complain of pain in her knees and hips, particularly with movement. A friction rub is heard on auscultation of her chest. An abnormality detected by which of the following serum lab oratory findings is most characteristic of the disease affecting this girl?

A) Antistreptolysin O antibody titer

B) Antinuclear antibody titer

C) Creatinine level

D) Rapid plasma reagin test

ANS: A

Female slide 33

a patient with heart murmur, what is the least expected finding?

A. aschoff bodies

B. fibrinous pericarditis

C. fused valves

D. diffuse fibrosis

ANS: A

Female slide 35

(murmur in chronic RHD , aschoff in acute)

Infective endocarditis

69- Which bacteria is most likely to cause infective endocarditis in IV drug users?

A. Staph. Auerus
B. hemolytic bacteria
C. Streptococcus
D. H. Influenza

Ans: A
Female slide 7

70- A patient with SLE has vegetations on his valves, what is the cause?
A. Immune complex deposition
B. Bacteria deposition
C. Abnormal CT
D. Atherosclerosis

Ans: A
Female slide 29

46- 15y came to emergency with fever 38.8 , fatigue, weight loss.Blood culture show patient have alpha hemolytic bacteria . few days ago she went to dentist . Fever still present in low grade with fatigue. What is the cause for her case ?

a. Acute Endocarditis
b. Subacute Endocarditis
c. Rheumatic fever
A. Libman-Sacks Endocarditis

Ans: B
(a- hemolytic-> subacute , staph -> acute)

56- when does mitral regrutiaon occur?

a. myxomatous mitral valve
b. Rheumatic Valvular Disease
c. Nonbacterial Thrombotic Endocarditis
d. Infective Endocarditis

Ans: A

Female slide 34

57- patient developed mucinous adenocarcinoma what will Vavilar disease occur in this condition?

a. NBTE
b. Infective Endocarditis
c. Myxomatous Mitral Valve
D. Libman-Sacks Endocarditis

Ans: A
Female slide 26

4- A man collapsed, CPR is done, he died, his history revealed that he had a myxomatous mitral valve, what is the possible cause to his death?
a. Rupture of valve leaflets or chordae tendineae
b. Ventricular arrythmia
c. Mural thrombus
D. Mitral valve stenosis

Ans: B
Female slide 40

48- Characteristics of subacute infective endocarditis
a. Rapid progression of infection
b. Affect deformed valves
c. Highly virulence organism
d. Significant morbidity & mortality even with antibiotics

Ans: B
Female slide 4

36 year old woman , blood cultures are positive for gram positive bacteria. Urine analysis show hematuria. An ecg shows a 1.5 cm vegetation on the mitral valve. Which of the following conditions is this patient most likely to develop?
A) cerebral arterial mycotic aneurysm
B) dilated cardiomyopathy
C) myxomatous mitral valve degeneration
D) pulmonary abscess

Ans: A

Female slide 9

A 71-year-old woman has had a 10-kg weight loss accompanied by severe nausea and vomiting of blood for the past 8 months. On physical examination, she is afebrile. Biopsy specimens obtained by upper gastrointestinal endoscopy show adenocarcinoma of the stomach. Which of the following cardiac lesions is most likely to be present in this woman?
A. Calcific aortic valvular stenosis
B. Constrictive pericarditis
C. Left ventricular mural thrombosis
D. Nonbacterial thrombotic endocarditis

Ans: A
Female slide 26

12 years old boy developed beta-hemolytic streptococcus pharyngitis which resolved spontaneously, however three weeks later he presented with a skin rush and his doctor hear murmur on physical examination, which of the following changes is least likely to be found in the heart of this boy at this stage of his disease?
A) Aschoff nodules in the myocardium
B) Fibrosis of the mitral valve with fusion of the commissures
C) Foci of fibrinoid necrosis in mitral valve
D) Fibrinous pericarditis
ANSWER: B

Pharmacology

Anti-arrhythmic drugs

1- A patient was given two doses of adenosine, but the condition remains. What is the best drug to give?
A. Amiodarone
B. Verapamil
C. Beta blockers
D. Digoxin

Answer : B or C (Verapamil or Beta blockers if adenosine doesn't work to treat acute paroxysmal supraventricular tachycardia).

2- A 36-year-old woman with a history of poorly controlled thyrotoxicosis and recurrent episodes of tachycardia with severe shortness of breath was admitted to the emergency room. Which of these medications is the most suited?
A. Esmolol
B. Quinidine
C. Amiodarone
D. Dopyramidine

Answer: A (Esmolol)

3- A patient came to the ER with ventricular tachycardia, tremors, dizziness, slurred speech, and disturbed hearing. What is the suitable drug?
A. Lidocaine
B. ACE inhibitor
C. Digoxin
D. Hydralazine

Answer: A

Female edit slide 23

4-Which drug acts on Na+ and K+ channels to control arrhythmia?
A. Beta blockers
B. Amiodarone
C. Lidocaine
D. Digoxin

Answer: B

Female edit slide 30

5-A patient had acute paroxysmal tachycardia and was given adenosine. What is the mechanism of action of this drug?
A. K+ channel opener
B. Ca2+ channel opener
C. Block Ca2+ channel
D. Block K+ channel

Answer: A

Female edit slide 35

6- A 65-year-old male's ECG shows supraventricular tachycardia. Which is most likely to cause this arrhythmia?
A. Atropine
B. Lidocaine
C. Digoxin
D. Adenosine
Answer: D

7- A patient with heart disease is treated with amiodarone. What should be monitored?
A. Heart conductivity and kidney function
B. Pulmonary and thyroid function
C. White blood cells
D. Liver and kidney function

Answer: B

Female edit slide 30

L5- Anti-anginal Drug:

1- A 47 year old man comes in with angina, he takes propranolol with nitrate. The combination of these drugs is to:
A. Prevent nitrate tachycardia
B. Induce propranolol bradycardia
C. Prevent nitrate vasodilator
D. Induce nitrate hypotension

Ans: A
Female slide 29

2- A 40 year old patient was prescribed sublingual nitroglycerin and isosorbide mononitrate after an acute anginal attack. Which of the following adverse effects is likely to occur?
a. Syncope
b. Methemoglobinemia
c. Postural Hypotension
d. Reflex bradycardia

Ans: C
Female slide 18

3- A woman was shopping at a mall, suddenly she had chest pain with vasospasm. Immediately she used nitroglycerin and it relieved her symptoms. What is the mechanism of action of nitrates to alleviate chest pain? (not exact wording)

A. Decrease pulmonary arterial pressure
B. Decrease O2 demand
C. Increase preload
D. Constrict coronary arteries

Ans: B
Female slide 15

4-Not a vasodilator but useful in preventing angina?

A) Hydralazine
B) Verapamil
C) Nifedipine
D) Propranolol

Ans: D

Female slide 20

5- The cellular effect when combined these 2 drugs (nitrites and sildenafil) is :

A. Decrease IP3
B. Phosphorylation of MlCK
C. Accumulation of c-GMP
D. Increase c-AMP levels

Ans: c

Female slide 18

6- All of these drugs are best indicated in these situations except :

A. B blockers / variant angina
B. Benign prostatic hypertrophy / selective a2 blocker
C. Diabetes / ACE inhibitors
D. Aspirin / unstable angina

Ans: A

Female slide 21

7- A male patient was given nitrate for his angina, he was non-compliant and took sildenafil for his sexual dysfunction, what can this lead to?

A. Acute MI
B. Hypertension
C.Bradycardia
D.Reflex tachycardia

Ans: A

Female slide 18

8- Which is the nitrate that is taken orally only?
A. Isosorbide dinitrate
B. Isosorbide mononitrate
C. Glyceryl trinitrate
D. Nitroglycerin patch

Ans: B
Female slide 17

Pharmacology: Antiplatelet and fibrinolytic drugs

1-What drug could be used with a woman going through an ischemic attack but is allergic to aspirin?

A. Ticlopidine
B. Clopidogrel
C. Warfarin
D. Heparin

Ans: B

Female Edit slide 17

2-Which of the following drugs inhibit IIb/IIIa receptor?

A. Abciximab **
B. Dipyridamole
C. Ticlopidine
D. Clopidogrel

Ans: A

Female Edit slide 21

3- **Which of the following drugs acts through irreversible inhibition of platelets?**

A. Aspirin
B. Celecoxib
C. Diclofenac
D. Acetaminophen

Ans: A

Female Edit **slide 8**

4- Which of the following drugs bind to glycoprotein receptor IIb/ IIIa to inhibit platelet aggregation?

A. Abciximab
B. Dipyridamine
C. Streptokinase
D. Clopidogrel

Ans : A
Female Edit slide 21

5- **A 60 year old male came to the emergency department with chest pain that started 2 hours ago. Laboratory tests showed he has a myocardial infarction; What medication should he take to return blood perfusion?**

A. Ticlopidine
B. Alteplase
C. Eptifibatide
D. Dipyridamol

Ans : B
Female Edit slide 29

6-**Which of the following antiplatelet drugs is primarily indicated for intermittent claudication?**

A. Cilostazol
B. Aspirin
C. Heparin
D. Warfarin

Ans :A

Female Edit slide 15

7- **A patient was hospitalized for an acute myocardial infarction received Abciximab. What is the mechanism of action and rational for using this drug?**

A. ADP receptors inhibitors
B. Glycoprotein IIb/IIIa inhibitors
C. Phosphodiesterase inhibitors
D. COX- 1 inhibitors

Ans :B
Female Edit slide 21

8-Which of the following drugs irreversibly inhibits platelet aggregation?

A.Clopidogrel
B.Abciximab
C.Salicylic acid
D. Ticlopidine

Ans: A
Female Edit slide 16

9- Mechanism of action of aspirin?

A. inhibits COX
B.Inhibits phosphodiesterase
C. Blocks the ADP receptor
D.Inhibits thrombin

Ans: A
Female Edit slide 10

10-Which drug should be given as soon as possible after an acute MI (<6hrs)?

A. Aspirin
B. Streptokinase

C. Metoprolol
D. Atorvastatin

Ans: B

Female Edit slide 29

Pharmacology: Drugs used in congestive heart failure

1-the mechanism of action of digitalis :
A. Block Ca and Na exchanger
B. Block Na k+ pump
C. Ca efflux
D. Increases The slope of SA node
E. Decreased calcium inflow
F. Stimulate beta-1 receptor

Ans: B
Edit female slide 23

2-Which of the following drugs have a good control on the heart rate and reduce mortality?
A. Beta blockers
B. Calcium Channel blockers
C. ACEI
D. ASRs

Ans: A
Edit female slide 19

3-Digitalis exert a positive inotropic effect because it :
A.Decrease Ca++ permeability
B.Inhibiting Na+ K+ pump
C. Activates the Na^+/K^+ pump
D. Increases K^+ influx

Ans: B
Edit female slide 23

5-Which of the following contraindicated to initiate beta blocker for congestive heart failure?
A. Recent decompensated HF
B. Ventricle ejection<40

C. Current digoxin use
D. Hypertension

Ans: A
Edit female slide 19

6.-A patient came in with digoxin toxicity into the ER, his potassium levels were normal,How do we manage this?

A. Digoxin Fab
B. Potassium salts
C. Activated charcoal
D. Sodium bicarbonate

Ans: A
Edit female slide 28

30) patient came with congestive heart failure + he take NSAIDs . He will
a- Cut it due to fluid retention
b- Useful because it augmented
c- contraindication in asthma
Answer :A

Pharmacology SDL

What is the mechanism of action of dapagliflozin ?
a- Inhibit SGLT2 (Na-glucose transport inhibitor)
b- positive effects on the cardiovascular system
c- angiotensin receptor blocker blocking the RAAS system
d- neprilysin inhibitor

Ans :A
(SDL)

What is the mechanism of action of Dapagliflozin which is a recently introduced drug for heart failure patients with reduced ejection fraction to reduce their mortality?
A. SGLT 2 Inhibition
B. Angiotensin receptor neprilysin inhibition
C. Angiotensin converting enzyme inhibition
D. Mineralocorticoid receptor antagonism

Ans :A
(SDL)

Medicine

Medicine Lecture 2: Heart failure

1- Which of the following is considered as a precipitating factor for Heart failure?

A. Change is sleeping habit
B. Exercise
C. CVD risk modification (Blood pressure control and monitoring BP)
D. Non-compliance to meds

Ans: D
(Not taking prescribed medications can lead to worsening heart failure and is considered a precipitating factor)

2-A 41 year old man presented to the outpatient clinic for A routine check-up . His examination reveals regular HR of 72 beat/min BP of 110/80 mmhg , downward and outward displacement of the cardiac apex . Faint S2, S4 gallop rhythm with systolic murmur at the aortic area and basal lung crepitation . His ECG shows LT ventricular concentric Hypertrophy, low ejection fraction of 38% with aortic transvalvular gradient of 48 mmhg. What is the most likely diagnosis?

A. Myocardial ischemia
B. Mitral valve prolapse (MVP)
C. Hypertension
D. Aortic stenosis complicated by HF

Ans: D

(The presence of a systolic murmur at the aortic area and an aortic transvalvular gradient of 48 mmHg strongly indicates aortic stenosis, which can lead to heart failure if left untreated. The patient has a low ejection fraction (38%), which indicates reduced heart function, commonly associated with heart failure.)

3- What is preferred to be a lower mortality in chronic congestive HF patients?

A. ACE
B. Diuretics
C. Antibiotics
D. NSAIDs

Ans: A

Female (Med 22) slide 46

4- The definition of heart failure with reduced ejection fraction increased all the following except:

A. Symptoms of HF
B. Signs of HF
C. EF>50
D. EF<40

Ans: C

Female (Med22) slide 11

5- Which of these are not clinical signs of Heart failure?

A. Orthopnea
B. Paroxysmal nocturnal dyspnea
C. Cough with yellow sputum
D. High heart and respiratory rate

Ans: C

Female (Med22) slide 20

6- 61 year old male suspected to have chronic heart failure, which of the following is recommended for diagnostic tests?

A. Insulin like growth factor
B. BNP
C. Urine collection
D.Hemoglobin A1c

Ans: B

Female (Med22) slide 31

7- What is a predisposing factor to heart failure?

A. Acute MI
B. Exercise
C. Weight gain
D. Healthy diet

Ans: A

Female (Med22) slide 19

8- Regarding left sided HF?

A. Congestive neck vein
B. Central vein pressure decreased
C. Right ventricle HF is more dangerous
D. Associated with orthopenea

Ans: D

(In left-sided heart failure, the left ventricle fails to pump blood efficiently, causing blood to back up in the lungs. When a person lies down, more blood returns to the heart, which further increases pulmonary congestion, leading to shortness of breath)

9- All the following is true about A heart failure except :

A. Heart failure can be caused by conditions like high blood pressure and coronary artery disease.

B. Heart failure may lead to fluid buildup in the lungs and other parts of the body.

C. Early diagnosis and treatment can improve the quality of life for patients with heart failure.

D. heart failure risk factors are irreversible

Ans: D

(certain risk factors for heart failure, such as age and genetic predispositions, are irreversible, however ,many others are modifiable including lifestyle factors like obesity, smoking)

10- Which of the following is not a risk factor for heart failure?

A.chronic liver disease

B.Diabetes mellitus

C.Hyperlipidemia

D.Hypertension

Ans: A

Female (Med22) slide 9

11- All of the following are symptoms of heart failure except:
A. Shortness of breath (SOB)
B. Paroxysmal nocturnal dyspnea
C. Fever ✅✅
D. Orthopnea

Ans: C

Female (Med22) slide 20

12- Patient came to the hospital and the doctor diagnosed with heart failure but the doctor didn't request ECG because?
A. ECG is not diagnostic of heart failure
B.ECG is expensive
C.ECG aggregates heart failure symptoms
D.ECG is invasive

Ans: A

Female (Med22) slide 28

13- Patient have symptoms and sign of heart failure what is the risk factor of coronary artery disease

A. Hypothyroidism
B. Neuropathy
C. Dyslipidemia
D. Encephalopathy

Ans :C

Female (Med22) slide 37

14- Patient came with CHF , what is the clinical manifestation of it

A. bilateral leg edema
B. fever
C. Severe headache
D. Nausea

Ans : A

Female (Med22) slide 19

L: Approach To Adult Congenital Heart Disease

68-aortic stenosis cause?

A-decrease after load
B-decreases vent pressure
C-concentric hyperatrophy

69- Regarding aortic valve stenosis:
A- increase pulmonary capillary pressure
B- produce faint S1
C- Will cause dilation of the left ventricle